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Understanding the U.S. Healthcare System

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Key Stakeholders:

Patient: Seeks medical care
Provider: Doctor, Hospital, Clinic providing services
Payer: Insurance company (e.g., Aetna, BCBS, Medicare)
Employer: Often pays part/all of insurance premium
Government: Regulates and funds (Medicare, Medicaid)

Types of Insurances:

Commercial Insurance: Provided by employer (Aetna, Cigna, etc.)
Government Plans:
Medicare: For elderly (65+) or disabled
Medicaid: For low-income individuals/families
Self-pay: No insurance – patient pays in full

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Introduction to Revenue Cycle Management (RCM)

Revenue Cycle Management (RCM) is the process that healthcare providers use to track patient care episodes from registration to final payment.

It combines clinical processes and administrative functions to ensure that healthcare organizations get paid for the services they deliver.

Why RCM is Important:

- Ensures financial health of healthcare providers
- Reduces errors and claim denials
- Improves cash flow and patient experience
- Promotes regulatory compliance

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Purpose

Ensures providers are reimbursed for services rendered.

Converts medical services into structured financial data.

Tracks and manages payments, balances, and denials.

Connects clinical treatment with insurance processing.

RCM Loop

Patient → Provider → Insurance → Payment/Denial → Patient Billing → Reporting

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US States and Abbreviations.

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50 USA states abbreviations list

SL	Name of States	Abbreviations	SL	Name of States	Abbreviations
1	Alabama	AL	26	Montana	MT
2	Alaska	AK	27	Nebraska	NE
3	Arizona	AZ	28	Nevada	NV
4	Arkansas	AR	29	New Hampshire	NH
5	California	CA	30	New Jersey	NJ
6	Colorado	CO	31	New Mexico	NM
7	Connecticut	CT	32	New York	NY
8	Delaware	DE	33	North Carolina	NC
9	Florida	FL	34	North Dakota	ND
10	Georgia	GA	35	Ohio	OH
11	Hawaii	HI	36	Oklahoma	OK
12	Idaho	ID	37	Oregon	OR
13	Illinois	IL	38	Pennsylvania	PA
14	Indiana	IN	39	Rhode Island	RI
15	Iowa	IA	40	South Carolina	SC
16	Kansas	KS	41	South Dakota	SD
17	Kentucky	KY	42	Tennessee	TN
18	Louisiana	LA	43	Texas	TX
19	Maine	ME	44	Utah	UT
20	Maryland	MD	45	Vermont	VT
21	Massachusetts	MA	46	Virginia	VA
22	Michigan	MI	47	Washington	WA
23	Minnesota	MN	48	West Virginia	WV
24	Mississippi	MS	49	Wisconsin	WI
25	Missouri	MO	50	Wyoming	WY

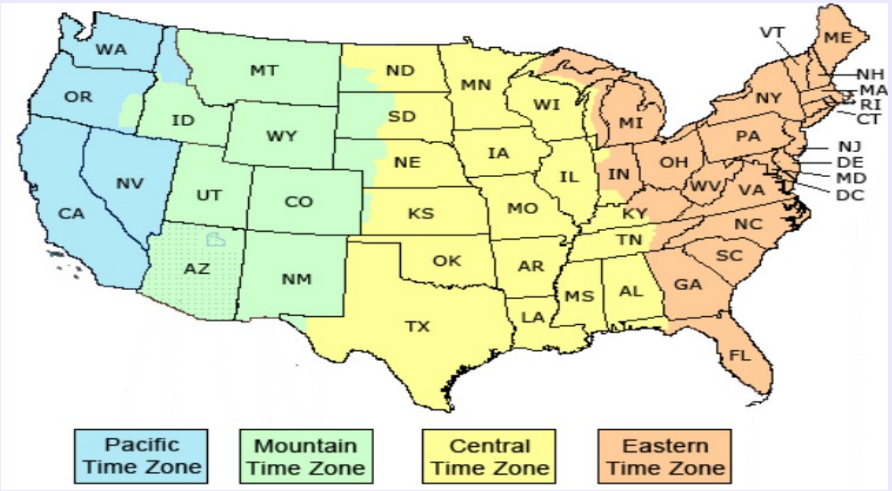
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Time Zone

A geographic region within which the same standard time is used

The United States uses four major time zones. From east to west they are Eastern Time Zone, Central Time Zone, Mountain Time Zone, Pacific Time Zone,.



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Time Zone

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Daylight Saving Time (DST) is the practice of moving clocks forward by one hour during summer months to make better use of daylight and extend evening daylight hours.

DST Schedule: It begins at 2:00 a.m. on the second Sunday in March and ends at 2:00 a.m. on the first Sunday in November.

Time Zone Changes: During DST, time zone abbreviations change, such as Eastern Standard Time (EST) becoming Eastern Daylight Time (EDT).

DST Observance: Most U.S. states observe DST, but Arizona (except the Navajo Nation), Hawaii, Puerto Rico, Guam, and the U.S. Virgin Islands do not.

Work Relevance: Understanding DST is important for scheduling client calls/meetings and for Healthcare BPO/RCM tasks such as submission deadlines, insurance eligibility checks, and claim follow-ups.

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Time Difference from India (IST)

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U.S. Time Zone	Standard Time (Nov–Mar)	Daylight Time (Mar–Nov)
Eastern (ET)	IST - 10.5 hours	IST - 9.5 hours
Central (CT)	IST - 11.5 hours	IST - 10.5 hours
Mountain (MT)	IST - 12.5 hours	IST - 11.5 hours
Pacific (PT)	IST - 13.5 hours	IST - 12.5 hours

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U.S. Numbering System – Quick Outline

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1. Number Scale (U.S. System)

- 1,000 = Thousand
- 1,000,000 = Million
- 1,000,000,000 = Billion
- 1,000,000,000,000 = Trillion

2. Comma Placement

- Commas are placed after every 3 digits from the right.
- Example: **1,000,000** (One Million)
- Avoid the Indian numbering format (e.g., **10,00,000**).

3. Currency Format

- Currency Symbol: **$** (U.S. Dollar)
- Standard Format: **$1,234.56**
- Two digits after the decimal represent cents.

4. Date Format

- Standard U.S. Format: **MM/DD/YYYY**
- Example: **07/22/2025** = July 22, 2025

5. Indian to U.S. Number Conversion

Indian Number	U.S. Equivalent
1 Lakh	100,000
10 Lakh	1 Million
1 Crore	10 Million
100 Crore	1 Billion

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U.S. Numbering System – Quick Outline

Phone Numbers

Format: **(XXX) XXX-XXXX**
U.S. country code: **+1**

1. Country Code

+1 → The international dialing code for the United States.

2. Area Code (First 3 digits)

Identifies a **specific geographic region** within the U.S.

Example:
212 = New York City
310 = Los Angeles
305 = Miami

Often shown in parentheses: **(212)**

3. Central Office Code (Next 3 digits)

Also called the **exchange code**

Refers to a **specific telephone exchange** within the area.

4. Line Number (Last 4 digits)

Identifies the **individual line or user**.

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Common U.S. vs British/ Indian Spelling Differences

British / Indian English	American English	Examples
-our	-or	colour → color favour → favor
-re	-er	centre → center metre → meter
-ise / -yse	-ize / -yze	organise → organize analyse → analyze
-ll	-l (in some cases)	enrolment → enrollment travelled → traveled
ae / oe	e	manoeuvre → maneuver oestrogen → estrogen
-ogue	-og	catalogue → catalog dialogue → dialog
defence / offence	defense / offense	defence → defense offence → offense
practice (noun/verb)	practice (noun) practise (verb)	In U.S., practice is used for both

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Examples in Context

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British / Indian Usage	American Usage
The colour of the sky is blue.	The color of the sky is blue.
He was taken to the centre.	He was taken to the center.
Please organise the files.	Please organize the files.
She has travelled a lot.	She has traveled a lot.

TIPS FOR FRESHER

- Always follow **U.S. spelling** when working with U.S.-based clients.
- Set MS Word or email tools to **"English (U.S.)"** for spell check.
- Be consistent: don't mix both styles in the same document.

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What Happens After a Doctor Visit

What happens once a patient leave the doctor's office in India/the US?

Why billing becomes complex in the US?

What role does insurance play in both countries?

Step	India (General Practice)	United States
1. Booking Appointment	Often walk-in or call clinic directly	Usually booked through online portals, apps, or via insurance network
2. Visiting the Doctor	Go to clinic/hospital; pay consultation upfront or after	Visit scheduled through insurance network; front desk verifies insurance
3. Payment	Pay directly to doctor or hospital (cash/card)	No immediate payment (except co-pay); doctor sends claim to insurance
4. Insurance Involvement	Optional – used only in bigger hospitals; insurance handled post-treatment	Mandatory – insurance is verified *before* treatment, claim submitted by provider
5. Billing System	Often paper-based or informal billing	Highly system-driven; every visit turns into a coded claim
6. Reimbursement	Reimbursement (if any) is done by patient	Insurance directly pays provider; patient may get a bill for balance

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What Happens After a Doctor Visit

Imagine This Scenario:

A patient visits a doctor in the U.S. for back pain. The doctor diagnoses and treats the patient. But who pays the doctor? - Not the patient directly — the claim goes to the insurance company!

That's **medical billing** in action — making sure the provider gets paid.

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RCM Terminologies

Term	Meaning
Provider	Doctor, hospital, or clinic giving medical care
Rendering Provider	The person who performed the service
Billing Provider	The one submitting the claim (could be same or different from rendering)
Referring Provider	The doctor who referred the patient to a specialist
Beneficiary	Patient covered under a health insurance plan
Guarantor	Person responsible for payment (usually the patient)
Payer	Insurance company paying for services
Coverage	Type of medical services included in patient's plan
Co-pay	Fixed amount patient pays per visit
Deductible	Amount patient must pay before insurance pays
Co-insurance	Shared cost between patient and insurance (e.g., 80/20)

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RCM Terminologies

Term	Meaning
Charge Entry	Entering services into billing system
Claim	A request sent to insurance for payment
CPT Code	Procedure code (what was done)
ICD Code	Diagnosis code (why it was done)
Modifier	Extra info about the procedure (e.g., repeated service)
Claim Scrubbing	Checking claim for errors before sending
Claim Rejection	Claim not accepted due to format or missing info
Claim Denial	Claim reviewed but not paid by insurance

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RCM Terminologies

Term	Meaning
Clearinghouse	System that checks & forwards claims to payers
837 File	Electronic format used to submit claims
EOB (Explanation of Benefits)	Document from insurance explaining payment
ERA (Electronic Remittance Advice)	Electronic version of EOB
Payment Posting	Recording payments and adjustments in system
AR (Accounts Receivable)	Money still owed to the provider
Patient Statement	Bill sent to patient for their balance
Follow-up	Action taken on unpaid or denied claims

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Match the Terms

Column A	Column B
1. EOB	A. Code for diagnosis
2. ICD-10	B. Prior approval for procedure
3. Modifier	C. Explains how claim was paid
4. Deductible	D. Extra info for CPT code
5. Authorization	E. Amount patient pays first

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Unscramble the Terms

AICML –

HIUGRONAESLCE –

RAE –

TORHOUAZITANOI –

DTUCDCIBELE –

MERIUMP –

TNEMYAP GINSOTP –

RPEIVORD –

SNECRUIAN –

OFMDEFISIR –

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HIPAA Compliance: What It Is & Why It Matters

What is HIPAA?

HIPAA stands for: Health Insurance Portability and Accountability Act (1996)A U.S. federal law that ensures the privacy, security, and confidentiality of a patient's health information (PHI – Protected Health Information)

Why Is HIPAA Important in RCM?

- Protects patient confidentiality
- Ensures secure handling of medical data (EHRs, claims, billing info)
- Required for all healthcare staff — including billing & BPO teams
- Violations can result in heavy fines and legal action

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HIPAA Compliance: What It Is & Why It Matters

What Must Be Protected (PHI)?

- Patient Name
- Date of Birth
- Medical Record Number
- Insurance Info
- Diagnoses & Treatment Details
- Any data that identifies the patient

Your Responsibility as an RCM Staff

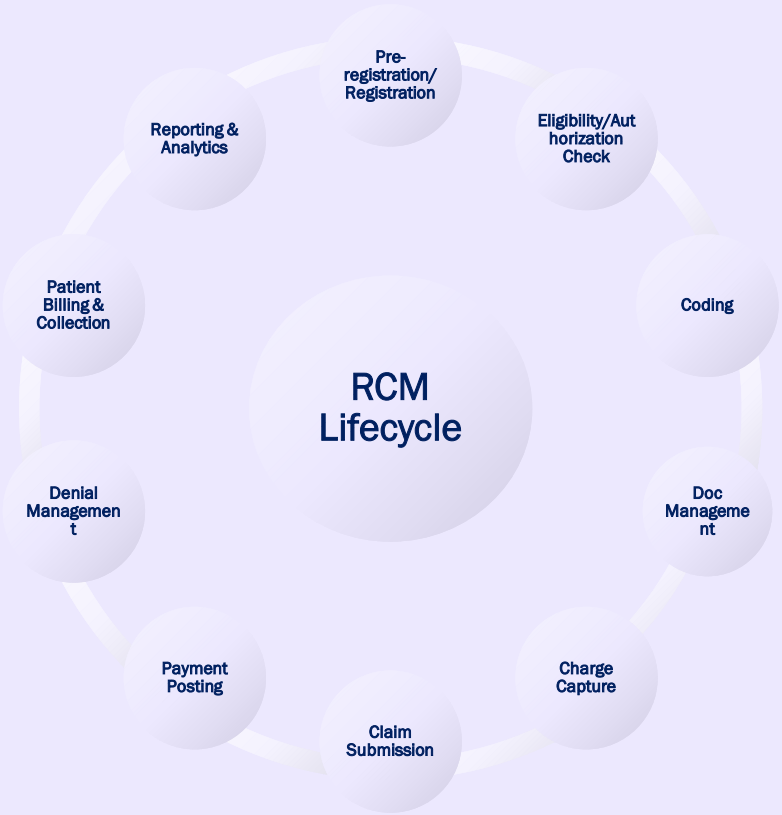
- Do **not share PHI** unnecessarily
- Use **secure systems** (no screenshots, downloads)
- **Log off** when leaving your system
- **Report** any accidental exposure or breach immediately

"HIPAA is not just a law — it's a responsibility we carry every day."

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The RCM Lifecycle



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Arrange the Steps in Order

A. Claim submission

B. Patient visit

C. Payment posting

D. Benefit Verification

E. Charge entry

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Main Stakeholders in RCM

- Patient
  - The recipient of medical services
  - Provides demographic and insurance info
  - Pays copay, coinsurance, or full bill (if uninsured)
- Provider
  - Doctor, Hospital, Clinic, or Healthcare Organization
  - Delivers care and initiates revenue cycle
  - Responsible for documentation and charge capture
- Front Office / Registration Staff
  - Collects patient details
  - Verifies insurance eligibility
  - Ensures correct information for billing

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Main Stakeholders in RCM

- Medical Coders
  - Translate diagnosis and procedures into ICD, CPT, HCPCS codes
  - Ensure coding accuracy and compliance
- Billing Team
  - Creates and submits claims to insurance
  - Ensures clean claim submission
- Payers (Insurance Companies)
  - Review and process claims
  - Make payment or issue denials
  - Examples: Aetna, Cigna, Medicare, Medicaid

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Main Stakeholders in RCM

- Accounts Receivable (AR) Team
  - Tracks payments
  - Works on denied/rejected claims
  - Follows up with insurance and patients
- Payment Posting Team
  - Posts payments received from insurance and patients
  - Reconciles accounts
  - Identifies underpayments or denials
  - Patient Collections
  - Communicates balances to patients
- Compliance & Audit Team
  - Ensure adherence to HIPAA, coding, and billing guidelines
  - Prevent fraud and abuse

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Patient Demographics

Patient Demographics refer to the basic identifying and contact information collected at the time of patient registration.

Key Patient Demographic Details:

Field	Description / Use
Full Name	Must match insurance records
Date of Birth (DOB)	Used to verify coverage eligibility
Gender	Relevant for certain procedures/tests
Address	Needed for communication & patient statements
Phone Number(s)	For contact and billing follow-up
Email ID	Optional, used for digital communication
Insurance Information	Plan name, member ID, group number
Social Security Number	Often used for patient identification (US only)
Guarantor Details	Who is financially responsible (if minor/dependent)
Emergency Contact	For patient safety during visit

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Patient Demographics

Why It Matters:

Wrong DOB ⟶ Claim Denied

Incorrect Insurance ID ⟶ Claim Rejected

Missing address ⟶ Patient won’t receive statement

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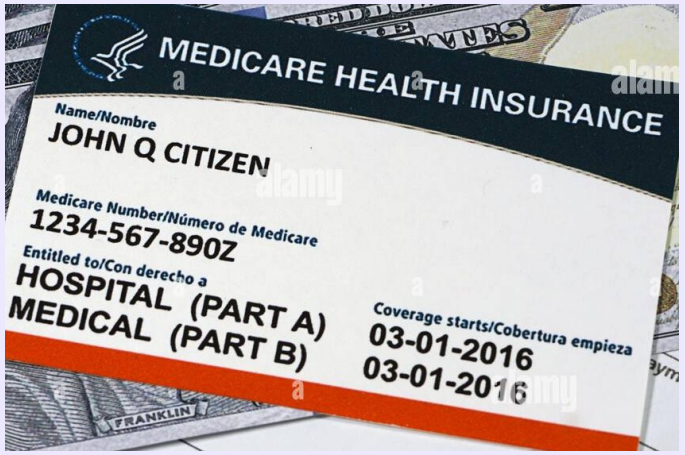
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Pre-registration & Registration

Pre-registration: Patient's basic info and appointment are collected

Registration: Demographics, insurance details, ID proof, consent

Errors here cause claim rejections later!



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Medical Records Management

Medical Records Management (MRM) involves the organization, storage, retrieval, and secure handling of patient health records — both digital and paper-based.

Why It Matters in RCM

- Ensures accurate documentation for coding and billing
- Supports medical necessity for claims
- Essential for appeals and audits
- Required for compliance (HIPAA, etc.)

Key Activities

- Reviewing patient charts for completeness
- Ensuring timely documentation by providers
- Uploading/scanning supporting documents (lab results, visit notes)

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Quiz

The patient's _____ and date of birth must exactly match what is listed with their insurance company.

_____ is the field where we record who is financially responsible for the patient, especially in the case of minors.

If the _____ is entered incorrectly, the claim may be denied due to incorrect coverage.

The _____ is used to contact the patient in case of billing issues or to send appointment reminders.

Patient demographics should be verified every _____ to avoid outdated or incorrect information.

_____ contact details are collected in case of emergencies during the patient's visit.

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Quiz

_____ records include referral letters, lab reports, prior authorizations, and insurance cards.

Before uploading a document into the system, it must be categorized correctly under the proper _____.

All uploaded documents should be _____-protected to ensure patient data privacy.

If a scanned document is blurry or incomplete, the _____ team must be alerted immediately to re-upload a clearer version.

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Eligibility & Benefits Verification

Check if patient is eligible for services under their insurance

Verify coverage, co-pay, deductible, authorization needs

Done before the visit to avoid unpaid claims

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Eligibility & Benefits Verification

Why It's Important:

- Prevents claim denials due to inactive or invalid insurance
- Identifies copay, deductible, coinsurance upfront
- Helps collect payment from the patient at the time of service
- Saves time and cost on rework or appeals

Key Information to Verify:

Field	What to Look For
Member ID & Group Number	Must match with patient info
Policy Status	Active or Terminated
Coverage Type	Medical, Dental, Vision, Mental Health, etc.
Effective & Expiry Dates	Must include date of service
Copay / Deductible Info	Understand upfront patient cost
Primary vs Secondary Payer	For coordination of benefits
Prior Authorization Needed?	Yes/No for specific services

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Medicare & Medicaid Eligibility Criteria

Medicare

- People age 65 or older
- Younger people with disabilities
- People with End Stage Renal Disease (permanent kidney failure requiring dialysis or transplant)

Medicaid

- Medicaid provides health coverage for some low-income people, families and children, pregnant women, the elderly, and people with disabilities
- Medicaid is Poor People Insurance
- Its also called as LAST RESORT.

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Understanding Authorization

Authorization (also called Pre-Authorization or Pre-Certification) is approval from the insurance company before a specific service or procedure is performed.

Usually required for:

- Surgeries
- Expensive imaging (e.g., MRI, CT)
- Specialty treatments or medications
- Inpatient admissions.

Why is Authorization Important?

- Ensures the service is medically necessary
- Prevents claim denials due to lack of pre-approval
- Helps providers understand coverage rules of the insurance plan
- Protects patients from unexpected costs

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Understanding Authorization

What Happens if Authorization is Missed?

- Claim gets denied — even if the service was valid
- Insurance refuses to pay
- Patient may become responsible for full cost
- Increases AR and rework for billing teams

Best Practices

- Always check payer rules before scheduling
- Maintain a tracker/log of authorization requests
- Get written proof of approval (Auth # or reference)
- Train front desk/scheduling teams to pre-check auth needs

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Quiz Time

1. What is the purpose of eligibility verification?
A. To see if the doctor is qualified
B. To confirm the patient's coverage and plan benefits
C. To check for past claims
D. To confirm the patient's address

Which one is true about prior authorization?
A. It is obtained after the service
B. It guarantees payment
C. It must be documented before certain services
D. It applies only to emergency care

What happens if eligibility is not verified before a visit?
A. Claim is processed faster
B. Patient always pays
C. The claim might be denied or delayed
D. Insurance gives retro coverage

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Quiz Time

What is typically included in an eligibility check?
A. Patient's last visit
B. Doctor's credentials
C. Plan status, deductible, copay, network info
D. Number of dependents

Medicare primarily covers which group?
A. Children under 10
B. Low-income families
C. Seniors age 65+ and people with disabilities
D. Pregnant women only

Which of these is a feature of Medicaid?
A. Uniform coverage in all states
B. Available only to the elderly
C. Jointly funded by state and federal governments
D. Available only to U.S. citizens

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Medical Coding

Diagnosis (ICD-10) and Procedure (CPT/HCPCS) codes are assigned

Must be accurate, specific, and compliant

Wrong code = denied or underpaid claim

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Diagnosis Code, CPT Code & Modifier

Diagnosis Codes (ICD Codes) - **ICD** stands for **International Classification of Diseases**

- Used to describe the reason a patient visited the doctor (illness, condition, or symptoms)
- Assigned by coders based on doctor's notes or medical documentation
- Follows the ICD-10-CM format in the U.S.
- Maintained by the WHO and CDC

CPT stands for **Current Procedural Terminology**

- Used to describe what the doctor did (exams, procedures, surgeries, etc.)
- Assigned to report the services performed
- Maintained by the American Medical Association (AMA)

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Diagnosis Code, CPT Code & Modifier

Modifiers

Modifiers are two-character codes (numbers or letters) added to a CPT code to provide additional information about the performed service — without changing the actual procedure code.

They help clarify what was done, how, and why, to ensure correct billing and payment.

When Are Modifiers Used? Eg : 25, TC

- Procedure was altered but not changed in definition
- Multiple procedures done in one visit
- Same service repeated by different providers or on different body parts
- Unusual circumstances (emergency, assistant surgeon, reduced services)

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Diagnosis Code and CPT Code

ICD-10-CM (Diagnosis Code) Format

- Alphanumeric
- 3 to 7 characters (First and Seventh Character are Alphabets)
- Decimal after the 3rd character
- Eg: S52.521A

CPT (Procedure Code) Format

- Numeric only
- 5 digits
- Eg: 99213

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Quiz – Identify which format is correct

S72.001A

9921A

E11.9

83112

72100

A25.

121.34

9a367

M2M.01

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Charge Entry

Services provided by the doctor are documented

These are converted into billable charges using CPT and ICD codes

Sources for Charge Entry

- Superbill/Encounter Form – Pre-defined template used by providers to mark services
- Medical Records (MR/EMR) – Review of physician documentation to confirm all services
- Pass-through Charges – Charges posted automatically by the system (e.g., based on EMR orders, lab tests)

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Key Checkpoints While Entering Charges

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- Verify Patient & Visit Details
- Match Diagnosis (ICD) and Procedure (CPT)
- Review Medical Records Thoroughly
- Units of Service
- Modifiers
- Follow Client-Specific Rules
- Pass-Through Charge Validation
- Avoid Duplication

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Common Errors to Avoid

Missing or wrong CPT/ICD codes

Incorrect provider or date of service

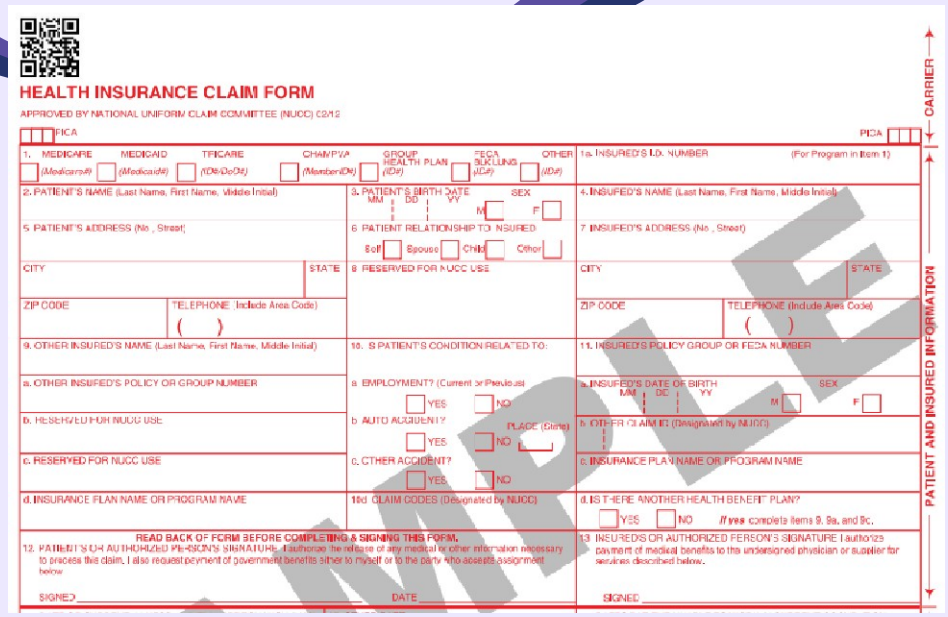
Billing services not documented in MR

Ignoring procedure bundling rules

Posting duplicate charges

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HEALTH INSURANCE CLAIM FORM

APPROVED BY NATIONAL UNIFORM CLAIM COMMITTEE (NUCC) 02/12

PICA

1. MEDICARE (Medicare#) MEDICAID (Medicaid#) TRICARE (ID#/DoD#) CHAMPVA (Member ID#) GROUP HEALTH PLAN (ID#) FECA BLK LUNG (ID#) OTHER (ID#)

1a. INSURED'S I.D. NUMBER (For Program in Item 1)

2. PATIENT'S NAME (Last Name, First Name, Middle Initial)

3. PATIENT'S BIRTH DATE MM DD YY SEX M F

4. INSURED'S NAME (Last Name, First Name, Middle Initial)

5. PATIENT'S ADDRESS (No., Street)

6. PATIENT RELATIONSHIP TO INSURED Self Spouse Child Other

7. INSURED'S ADDRESS (No., Street)

CITY STATE

8. RESERVED FOR NUCC USE

CITY STATE

ZIP CODE TELEPHONE (Include Area Code) ( )

ZIP CODE TELEPHONE (Include Area Code) ( )

9. OTHER INSURED'S NAME (Last Name, First Name, Middle Initial)

10. IS PATIENT'S CONDITION RELATED TO:

11. INSURED'S POLICY GROUP OR FECA NUMBER

a. OTHER INSURED'S POLICY OR GROUP NUMBER

a. EMPLOYMENT? (Current or Previous) YES NO

a. INSURED'S DATE OF BIRTH MM DD YY SEX M F

b. RESERVED FOR NUCC USE

b. AUTO ACCIDENT? YES NO PLACE (State)

b. OTHER CLAIM ID (Designated by NUCC)

c. RESERVED FOR NUCC USE

c. OTHER ACCIDENT? YES NO

c. INSURANCE PLAN NAME OR PROGRAM NAME

d. INSURANCE PLAN NAME OR PROGRAM NAME

10d. CLAIM CODES (Designated by NUCC)

d. IS THERE ANOTHER HEALTH BENEFIT PLAN? YES NO If yes, complete items 9, 9a, and 9c.

READ BACK OF FORM BEFORE COMPLETING & SIGNING THIS FORM.

12. PATIENT'S OR AUTHORIZED PERSON'S SIGNATURE I authorize the release of any medical or other information necessary to process this claim. I also request payment of government benefits either to myself or to the party who accepts assignment below.

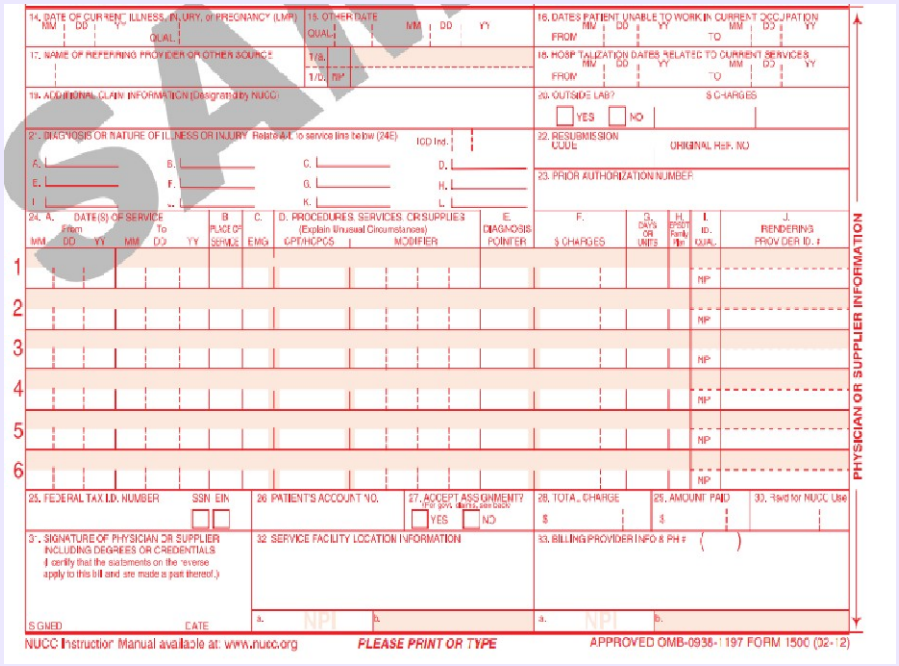
SIGNED DATE

13. INSURED'S OR AUTHORIZED PERSON'S SIGNATURE I authorize payment of medical benefits to the undersigned physician or supplier for services described below.

SIGNED

CARRIER

PATIENT AND INSURED INFORMATION



14. DATE OF CURRENT ILLNESS, INJURY, or PREGNANCY (LMP) MM DD YY QUAL.

15. OTHER DATE QUAL. MM DD YY

16. DATES PATIENT UNABLE TO WORK IN CURRENT OCCUPATION MM DD YY FROM TO

17. NAME OF REFERRING PROVIDER OR OTHER SOURCE

17a.

17b. NPI

18. HOSPITALIZATION DATES RELATED TO CURRENT SERVICES MM DD YY FROM TO

19. ADDITIONAL CLAIM INFORMATION (Designated by NUCC)

20. OUTSIDE LAB? YES NO $ CHARGES

21. DIAGNOSIS OR NATURE OF ILLNESS OR INJURY Relate A-L to service line below (24E) ICD Ind.

A. B. C. D. E. F. G. H. I. J. K. L.

22. RESUBMISSION CODE ORIGINAL REF. NO.

23. PRIOR AUTHORIZATION NUMBER

24. A. DATE(S) OF SERVICE From MM DD YY	To MM DD YY	B. PLACE OF SERVICE	C. EMG	D. PROCEDURES, SERVICES, OR SUPPLIES (Explain Unusual Circumstances) CPT/HCPCS	MODIFIER	E. DIAGNOSIS POINTER	F. $ CHARGES	G. DAYS OR UNITS	H. EPSDT Family Plan	I. ID. QUAL.	J. RENDERING PROVIDER ID. #
1											NP
2											NP
3											NP
4											NP
5											NP
6											NP

25. FEDERAL TAX I.D. NUMBER SSN EIN

26. PATIENT'S ACCOUNT NO.

27. ACCEPT ASSIGNMENT? (For govt. claims, see back) YES NO

28. TOTAL CHARGE $

29. AMOUNT PAID $

30. Rsvd for NUCC Use

31. SIGNATURE OF PHYSICIAN OR SUPPLIER INCLUDING DEGREES OR CREDENTIALS (I certify that the statements on the reverse apply to this bill and are made a part thereof.)

SIGNED DATE

32. SERVICE FACILITY LOCATION INFORMATION

a. NPI b.

33. BILLING PROVIDER INFO & PH # ( )

a. NPI b.

NUCC Instruction Manual available at: www.nucc.org PLEASE PRINT OR TYPE APPROVED OMB-0938-1197 FORM 1500 (02-12)

PHYSICIAN OR SUPPLIER INFORMATION

SAMPLE

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Claim Creation and Submission

Combine patient, insurance, charge, and coding details

Claims created using billing software (CMS 1500 - 837 format for electronic claims)

Submitted to clearinghouse or directly to payer

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Claim Form & Clearinghouse in RCM

Claim Forms - In the U.S. healthcare system, services must be billed to insurance companies using standard claim forms — either electronically (EDI) or on paper.

CMS-1500 (HCFA) Form

- Also called the HCFA form
- Used to bill professional services (doctors, clinics, private practices)
- Filed by physicians, nurse practitioners, therapists, etc.
- Contains up to 6 diagnosis codes, procedure codes, modifiers, provider/patient details

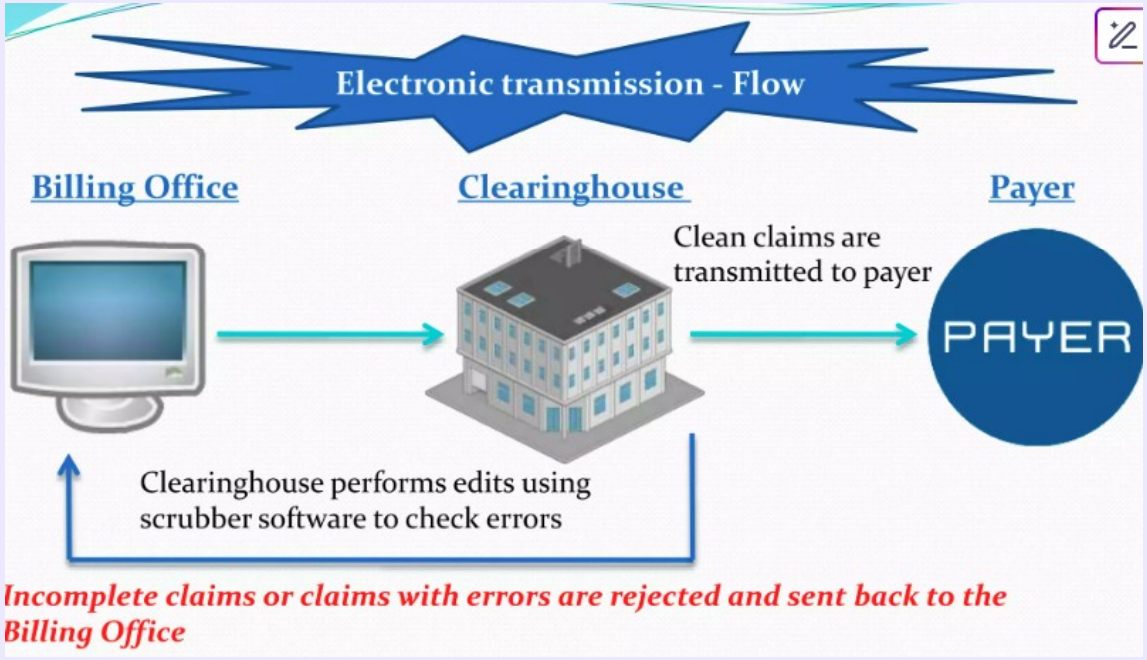
Clearinghouse

- A middle layer between providers and insurance companies
- Checks claims for errors and standard formatting (scrubbing)
- Converts claim data into HIPAA-compliant EDI format (837)
- Forwards cleaned claims to the correct payer (insurance company)

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Function of Clearing House



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Payment Posting

Payer sends EOB (Explanation of Benefits) or ERA (Electronic Remittance Advice)

Payments and adjustments are posted in the system

Patient balance is calculated

Types of Payments Received

- Insurance
- Patient

Types of Payment Posting Entries

Entry Type	Description
Payment	Amount paid by payer or patient
Adjustment	Amount not allowed by payer (contractual write-off)
Denial	Claim denied by insurance; needs follow-up
Refunds/Overpayments	Excess collected and needs to be returned or held
Patient Responsibility	Co-pay, deductible, coinsurance to be collected from patient

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EFT vs. Check Payments

Criteria	EFT (Electronic Funds Transfer)	Check Payment
Speed	1–2 business days	5–10 business days
Security	Encrypted and direct bank-to-bank	Risk of loss, theft, or delay
Posting	Can be linked with ERA for auto-posting	Requires manual entry
Tracking	Easier to reconcile with remittance	Delayed bank deposits and unclear remits
Cost to process	Lower (less paper handling)	Higher (mailing, manual handling)

Why EFT is better

- Faster payment
- Linked with ERAs for automation
- No physical paperwork or delays
- Fewer Delays

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ERA Sample

MEDICARE B
J1 MAC – PALMETTO GBA
PO BOX 1051
AUGUSTA, GA 30903-1051
866-931-3901

MEDICARE
REMITTANCE
NOTICE

ONCOLOGY WORLD MEDICINE
4935673 L STREET
RENO, NV 89511-2418
946-342-2585

PROVIDER #: V36205
PAGE #: Page 1 of 1
DATE: 05/03/2010
CHECK/EFT 881138051

REND PROV	SERV DATE	POS	NOS	PROC	MODS	BILLED	ALLOWED	DEDUCT	COINS	GRP/RC-AMT	PROV PD
NAME MANNING, EMMETT			HIC 586774830K			ACCT 4722X890127595		ICN 2210109179530		ASG Y	MOA MA18 MA01
	04/14/2010	11	1	99205		431.00	193.15	0.00	38.63	CO-45 237.85	154.52
PT RESP 38.63				CLAIM TOTALS		431.00	193.15	0.00	38.63	237.85	154.52
ADJ TO TOTALS:	PREV PD		0.00	INTEREST		0.00	LATE FILING CHARGE		0.00	NET	154.52
CLAIM INFORMATION FORWARDED TO: BLUE CROSS BLUE SHIELD											

GLOSSARY: GROUP, REASON, MOA, REMARK AND ADJUSTMENT CODES

MA01	If you do not agree with what we approved for these services, you may appeal our decision. To make sure that we are fair to you, we require another individual that did
45	45-Charges exceed your contracted/ legislated fee arrangement.
MA18	The claim information is also being forwarded to the patient's supplemental insurer. Send any questions regarding supplemental benefits to them.
CO	Contractual Obligations

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EOB Sample

Explanation of Benefits (EOB)

Customer Service: 1.800.123.4567

Statement Date: XX/XX/XXXX
Document Number: XXXXXXXXXXXXXX

Member Name: John Smith
Address: 1234 Main Street
City, State, Zip: Cleveland, OH 44115

THIS IS NOT A BILL

Subscriber number: XXXXXXXXXX ID: XXXXXXXXX Group: ABCDE Group Number: XXXXXXX

Patient Name:
Date Recieved:

Provider:
5 Payee:

Claim number: XXXXXXXXXX
Date Paid: XX/XX/XXXX

Claim Detail				2 Provider Charges		3 Your Responsibility			4 Total Claim Cost		
Line #	Service Date	1 Service Description	Claim Status	Provider Charges	Allowed Charges	Co-pay	Deductible	Co-Insurance	Paid by Insurer	6 What you owe	7 Remark Code
1	1/1/2015	Medical care	Paid	$41.50	$3.15	$0.00	$0.00	$0.00	$3.15	$0.00	PDC
2	1/1/2015	Medical care	Paid	$375.00	$118.15	$35.00	$0.00	$0.00	$85.50	$35.00	PDC
			Total	$416.50	$121.30	$35.00	$0.00	$0.00	$88.65	$35.00	

Remark Code: PDC - Billed amount is higher than the maximum payment insurance allows. The payment is for the allowed amount.

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Key Terminologies in EOB

Understanding insurance terms helps you interpret **eligibility, benefits, patient responsibility**, and **billing outcomes** correctly.

Term	What It Means
Premium	Amount paid (usually monthly) by the patient or employer to keep the insurance active — like a subscription fee.
Deductible	Fixed amount the patient must pay annually before the insurance starts paying (e.g., $1,500/year).
Co-pay	Fixed fee paid at the time of visit (e.g., $30 for a doctor's visit), regardless of total cost.
Co-insurance	Patient's share of the cost after deductible is met (e.g., insurance pays 80%, patient pays 20%).
Out-of-Pocket Maximum	Maximum the patient has to pay in a year (includes deductible, co-pays, co-insurance). After that, insurance pays 100%.
In-Network Provider	A doctor or facility contracted with the insurance company to provide services at a lower cost.
Out-of-Network Provider	Not contracted with insurance — patient may pay more or full cost.
Allowed Amount	Maximum amount insurance agrees to pay for a covered service.
Patient Responsibility	The portion of the bill the patient must pay — includes deductible, co-pay, co-insurance, non-covered services.

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Example

Total Bill: $200

Deductible: $100 left

Co-insurance: 20%

Allowed Amount: $180

➡☐ Patient Pays:

$100 deductible

20% of remaining $80 = $16

➡☐ Total Patient Responsibility: $116

➡☐ Insurance Pays: $64

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Riddle

I follow the claim, but I’m not the bill.
I explain your money, with codes I fill.
I’m sent with your pay, electronically real.
What am I? ----------

I take away what you can’t collect,
I’m neither denial nor disrespect.
Contractual rules are where I stay,
I shrink the bill in a legal way.
What am I? ----------

I move money without a check,
No envelope, no mailroom deck.
Quick and quiet, I land with flair,
Right into your billing software.
What am I? ----------

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Denial Management

Process of identifying, analyzing, correcting, and preventing claim denials received from insurance companies.

Denial - A **denial** is a response from the payer stating that **a claim or service will not be reimbursed.** This differs from a **rejection**, which is a claim not accepted for processing due to format issues.

Types of Denials

Type	Meaning
Hard Denials	Cannot be corrected or appealed (e.g., untimely filing)
Soft Denials	Can be corrected and resubmitted (e.g., coding errors, missing info)
Preventable Denials	Caused by errors in charge entry, coding, eligibility
Non-Preventable Denials	Related to payer policies (e.g., non-covered service)

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Common Reasons for Denials

- Incorrect patient details (name, DOB, insurance ID)
- Eligibility not verified before service
- Missing or invalid CPT/ICD codes
- Duplicate claims
- Lack of authorization or referral
- Late submission (beyond timely filing limit)
- Bundling/edit rules violations
- Modifier errors or missing documentation

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Best Practices to avoid Denials

Verify eligibility before service

Use claim scrubbers and edits before submission

Educate coding and charge entry teams

Monitor top denial reasons monthly

Appeal denials promptly and correctly

Maintain payer guidelines and updates

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Patient Billing & Collections

Patients are billed for their part (co-pay, deductible, etc.)

Follow-up if not paid on time

Statements, calls, payment plans, collections

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Reporting & Analytics

Track key RCM metrics:

- Clean claim rate
- Days in A/R
- Denial rate
- Collection rate
- Dashboards help identify issues and drive improvements

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Case Study

A patient came in for a flu shot. Insurance wasn't verified. Wrong code used. Claim denied.

A patient had a routine visit, but the provider did not document the service fully. The claim was denied due to lack of medical necessity.

Patient was scheduled for a procedure that required prior authorization, but it was not obtained. The claim was denied.

Patient came for a follow-up visit and the same service was billed again by mistake. The claim was denied as a duplicate.

The provider submitted claims under an old tax ID that was no longer active with the payer. The claim was denied.

- Which steps failed?
- How should this be fixed?
- What can we do to prevent this?

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RCM Vocabulary: Doctors, Payers & Plans

What is a Doctor Called?

- Rendering Provider, Treating Doctor - The provider who performed the service
- PCP (Primary Care Provider) - They are usually the first point of contact and may refer patients to specialists if needed.
- Referring Provider - The doctor who referred the patient
- Supervising Provider - A provider overseeing the service

Types of Insurance

- Commercial (e.g., Aetna, Cigna)
- Government (Medicare, Medicaid)
- Workers' Compensation
- Auto Insurance
- Tricare / VA

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RCM Vocabulary: Doctors, Payers & Plans

Common Insurance Companies

United Healthcare

Anthem Blue Cross

Cigna

Aetna

Humana

Medicare

Medicaid

BCBS (Blue Cross Blue Shield)

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RCM Vocabulary: Doctors, Payers & Plans

Types of Health Insurance Plans

Plan Type	PCP Needed	Referrals Needed	Out-of-Network	Cost
HMO	Yes	Yes	No	Low
PPO	No	No	Yes	High
EPO	No	No	No	Medium
POS	Yes	Yes	Yes (with referral)	Medium
Medicare	Optional	No	Yes (varies)	Varies
Medicaid	Yes	Varies	Varies	Free/Low

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RCM Vocabulary: Identifiers & Codes

Term	Full Form / Meaning	Used For
NPI	National Provider Identifier	A unique 10-digit ID for healthcare providers; required on all claims
TIN	Tax Identification Number (also called EIN)	Identifies a billing entity or organization for tax and billing purposes
EIN	Employer Identification Number	Same as TIN; issued by IRS to businesses
SSN	Social Security Number	Personal ID for individuals (not used in claims)
Taxonomy Code	Provider Specialty Classification Code	Specifies the provider's specialty (e.g., 207Q00000X for Family Medicine)
PTAN	Provider Transaction Access Number	Legacy Medicare ID (used internally by Medicare)
CLIA Number	Clinical Laboratory Improvement Amendments Number	Required for labs performing diagnostic testing
UPIN	Unique Physician Identification Number (retired)	Was used before NPI; now obsolete
Group NPI	NPI assigned to a group practice	Used when billing as a clinic or multi-provider practice
Individual NPI	NPI assigned to a single doctor or provider	Used when billing by individual providers
Payer ID	Unique ID assigned to each insurance company	Used for routing claims through clearinghouses

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Key Takeaways

Understanding the Healthcare System

- U.S. healthcare billing is complex and insurance-driven
- Accurate billing ensures that providers get paid and patients are billed fairly

Revenue Cycle Management (RCM) Basics

- RCM is the end-to-end process from scheduling to final payment
- Includes: Eligibility check → Charge entry → Claim submission → Payment posting → Denial management

Key Forms & Systems

- CMS-1500 (HCFA): For professional billing
- UB-04: For institutional billing
- Clearinghouse: Checks, formats, and routes claims to payers
- ERA/EOB: Documents that explain payment and denial info

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Financial Terms to Remember

- Premium: Monthly insurance fee
- Deductible: What patient pays before insurance starts
- Co-pay/Co-insurance: Patient's share per visit/service
- Out-of-pocket Max: Patient's total yearly limit

Charge Entry & Modifiers

- Always review superbill & medical records to capture full services
- Use correct CPT, ICD, and Modifiers (25, 59, etc.)
- Avoid duplicates and mismatches

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Key Takeaways

Payment Posting

- Record insurance and patient payments accurately
- Differentiate between payments, adjustments, and denials
- Prefer EFT + ERA over checks for speed and automation

Denial Management

- Denials ≠ end of payment
- Categorize → Correct → Resubmit/Appeal
- Track top denial trends and fix the root cause

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Best Practices Across the Cycle

Always verify eligibility before services

Use claim scrubbers and coding validations

Document everything — if it's not in the chart, it wasn't done!

Follow **payer-specific rules** and updates

Keep credentialing, tax IDs, and NPI records updated

Educate & empower continuously

Ultimate Goal of RCM

To ensure **timely, accurate, and complete reimbursement** while reducing denials, rework, and patient dissatisfaction.

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Wrap up Quiz

Section 1: Basics & Terminologies

1. What does RCM stand for?
A. Revenue Cycle Model
B. Revenue Collection Management
C. Revenue Cycle Management
D. Reimbursement Claim Management

2. True or False:
The CMS-1500 form is used for hospital billing.

3. Which of the following is a payer?
A. Patient
B. Clearinghouse
C. Insurance company
D. Provider

4. What is a premium in health insurance?
A. The amount insurance pays the provider
B. The patient's responsibility at each visit
C. The monthly fee paid to maintain insurance coverage
D. A penalty for denied claims

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Wrap up Quiz

Section 2: Eligibility, Charge Entry & Modifiers

5. What happens if eligibility is not verified before a patient visit?
A. Claim gets paid faster
B. Patient pays less
C. Claim may be denied or delayed
D. Nothing happens

6. True or False:
Modifier 25 is used when a procedure is repeated by a different provider.

7. What is the main purpose of modifiers in CPT coding?
A. To change the diagnosis
B. To indicate additional information about the service
C. To cancel a claim
D. To bypass insurance rules

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Wrap up Quiz

Section 3: Payment & Denials

8. Which of these is the fastest and most secure payment method from payers?
A. Cash
B. EFT (Electronic Fund Transfer)
C. Paper Check
D. PayPal

9. What is an ERA?
A. Patient payment form
B. Eligibility Request Authorization
C. Electronic Remittance Advice
D. Emergency Referral Authorization

10. True or False:
A claim denied due to missing authorization can be resubmitted after obtaining retro authorization.

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Wrap up Quiz

Section 4: Denials & Case Review

11. A claim is denied for “duplicate service.” What should you do first?
A. Appeal immediately
B. Rebill without changes
C. Check if the service was billed twice
D. Change the CPT code

12. Case: A patient visit was denied due to invalid DOB. Which team should be alerted?
A. Coding
B. Eligibility
C. Front Desk/Registration
D. Clearinghouse

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Wrap up Quiz

Section 5: Workflow & Best Practices

13. Arrange the steps in RCM in the correct order:

Payment Posting
Claim Submission
Charge Entry
Eligibility Verification

14. What is the role of a clearinghouse?

A. Sends payment to providers
B. Scrubs and routes claims to payers
C. Audits medical records
D. Issues insurance policies

15. True or False:

Denial management is only needed when the claim is rejected before submission.

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Wrap up Quiz

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Section 6: Terminologies

16. What does NPI stand for?

A. National Patient Identifier
B. National Provider Identifier
C. New Provider Insurance
D. Network Physician Identifier

17. What is the role of a Rendering Provider?

A. Reviews claims after payment
B. Refers the patient to another doctor
C. Supervises a nurse
D. Performs the actual service/treatment

18. Who is the Payer in RCM?

A. The patient's family
B. The government agency only
C. The entity that pays for the medical service
D. The provider's employer

19. What is a PCP?

A. Post Claim Process
B. Primary Care Provider
C. Patient Claim Payment
D. Provider Clearinghouse Portal

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Thank you

🎓Introduction to Revenue Cycle Management Session Complete

Questions or Clarifications?

Feel free to ask — no question is too small! We're here to help you understand and grow.

"Learning never exhausts the mind — it only fuels your success."

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